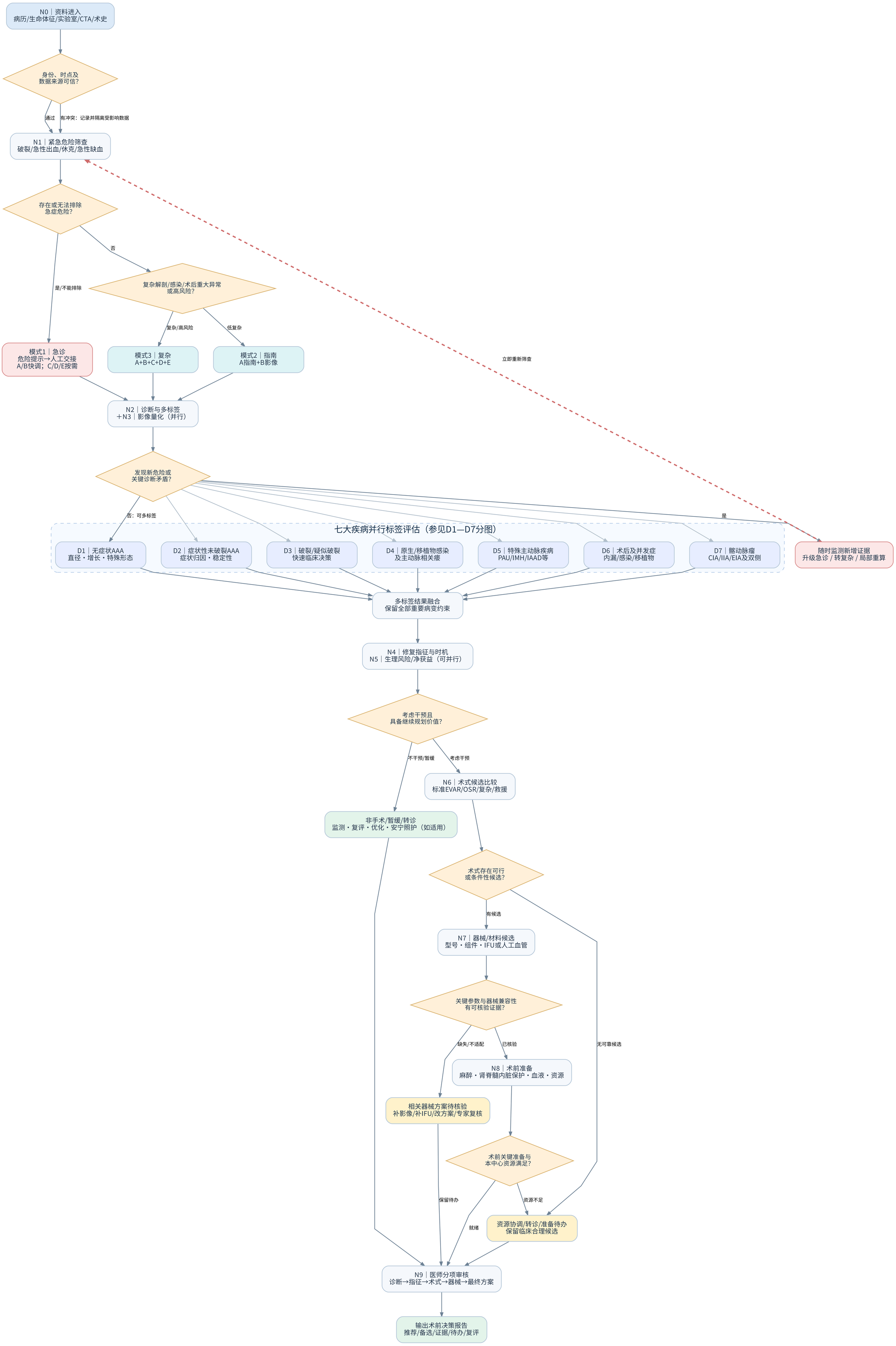
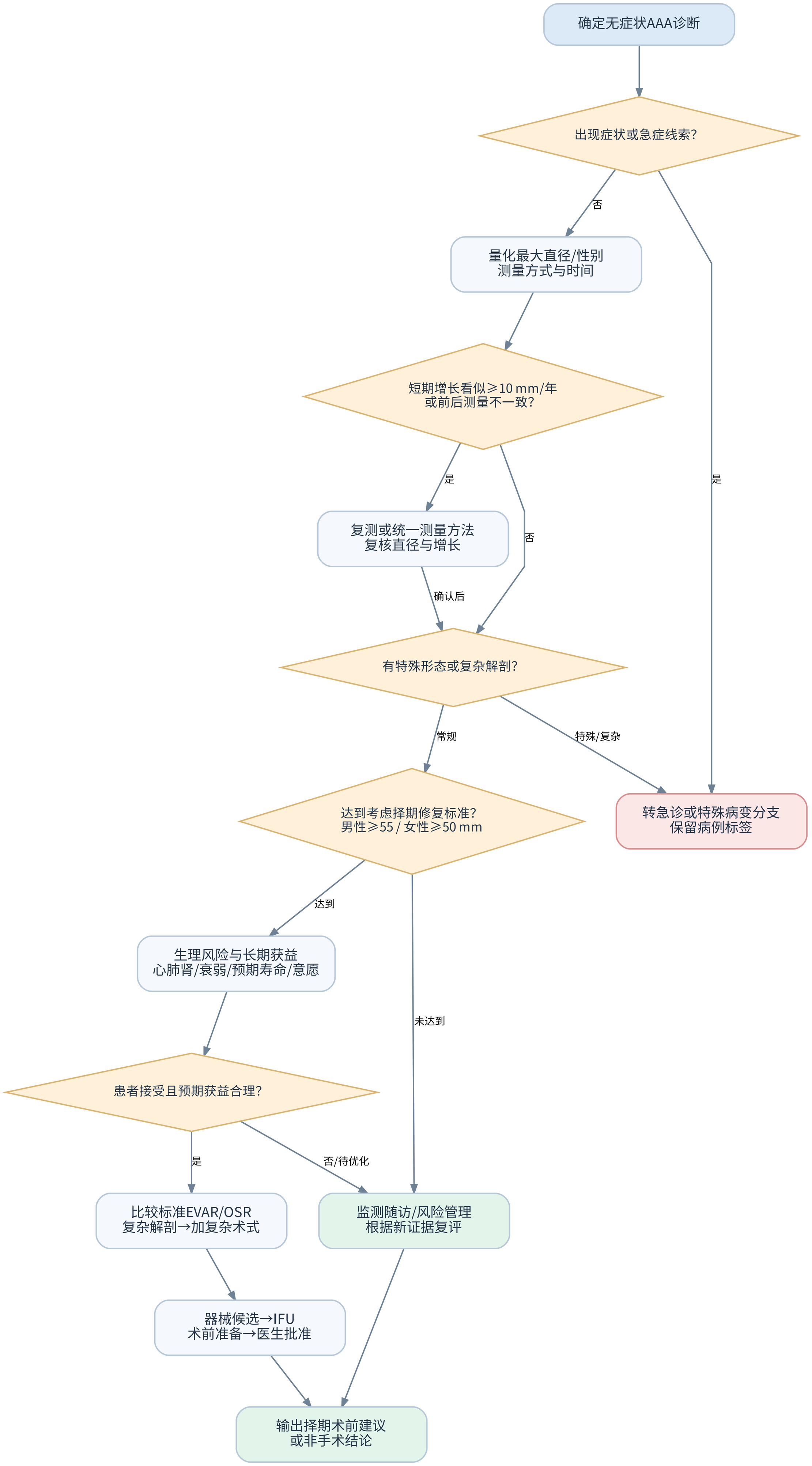
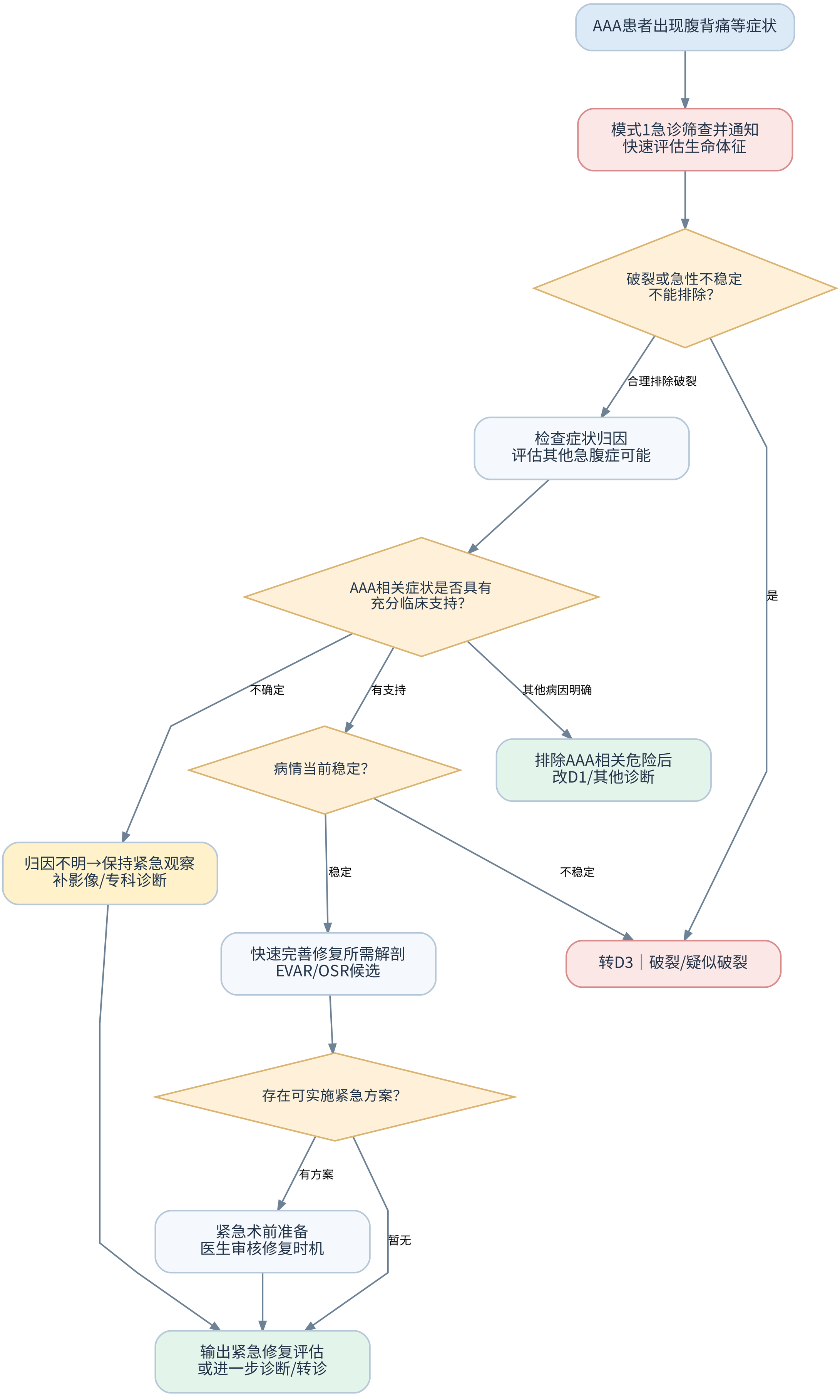
VascGuide V1.1｜详细术前决策总路线



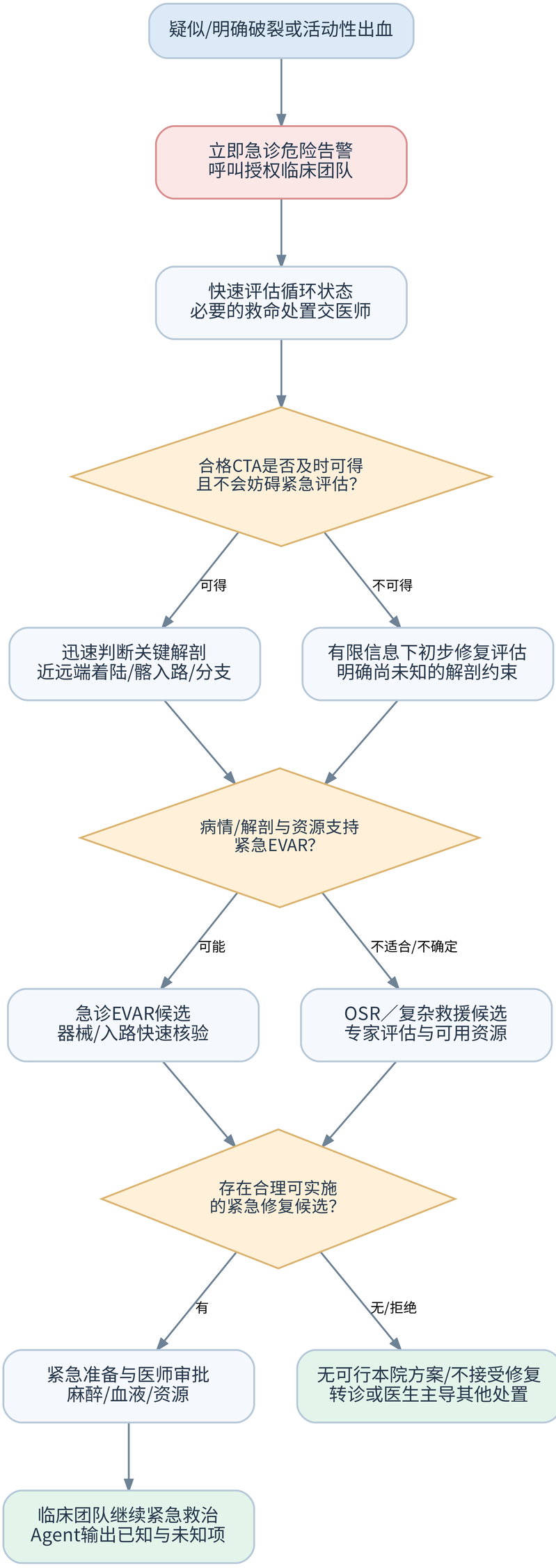
D1｜无症状腹主动脉瘤详细路径



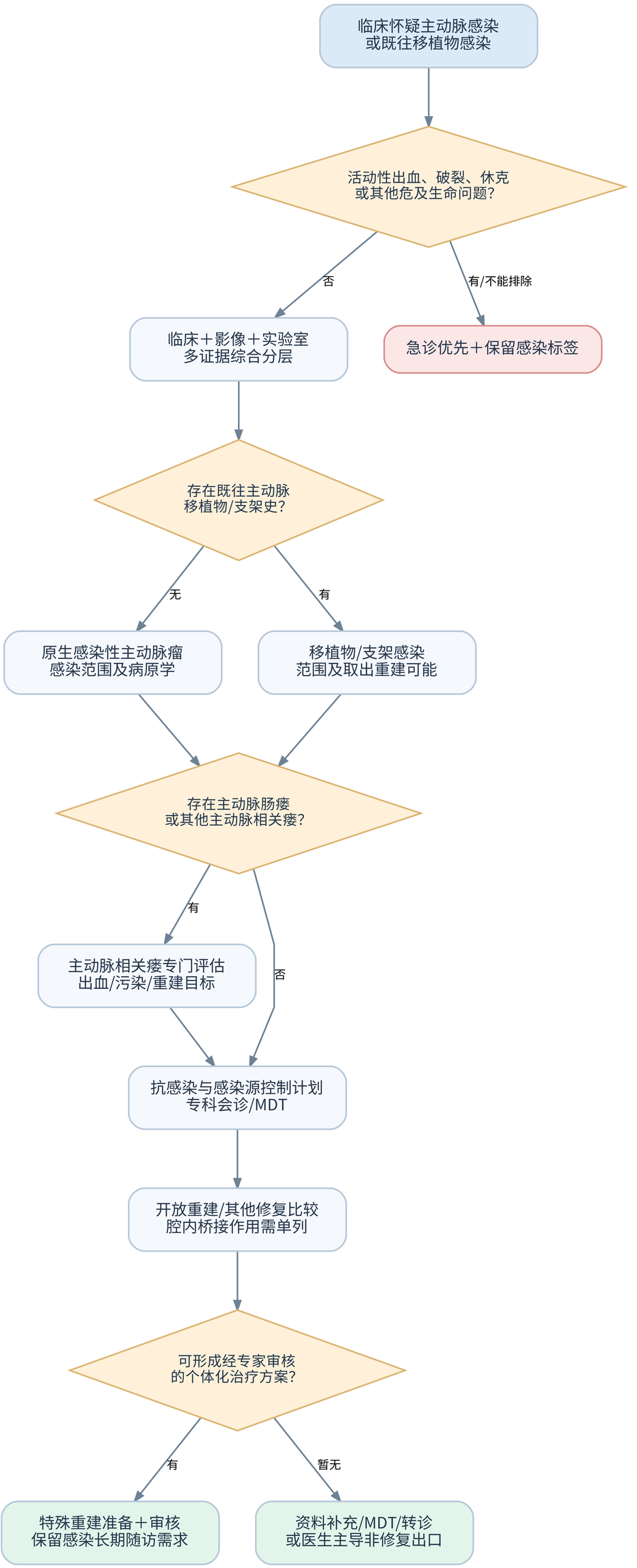
D2｜症状性未破裂AAA详细路径



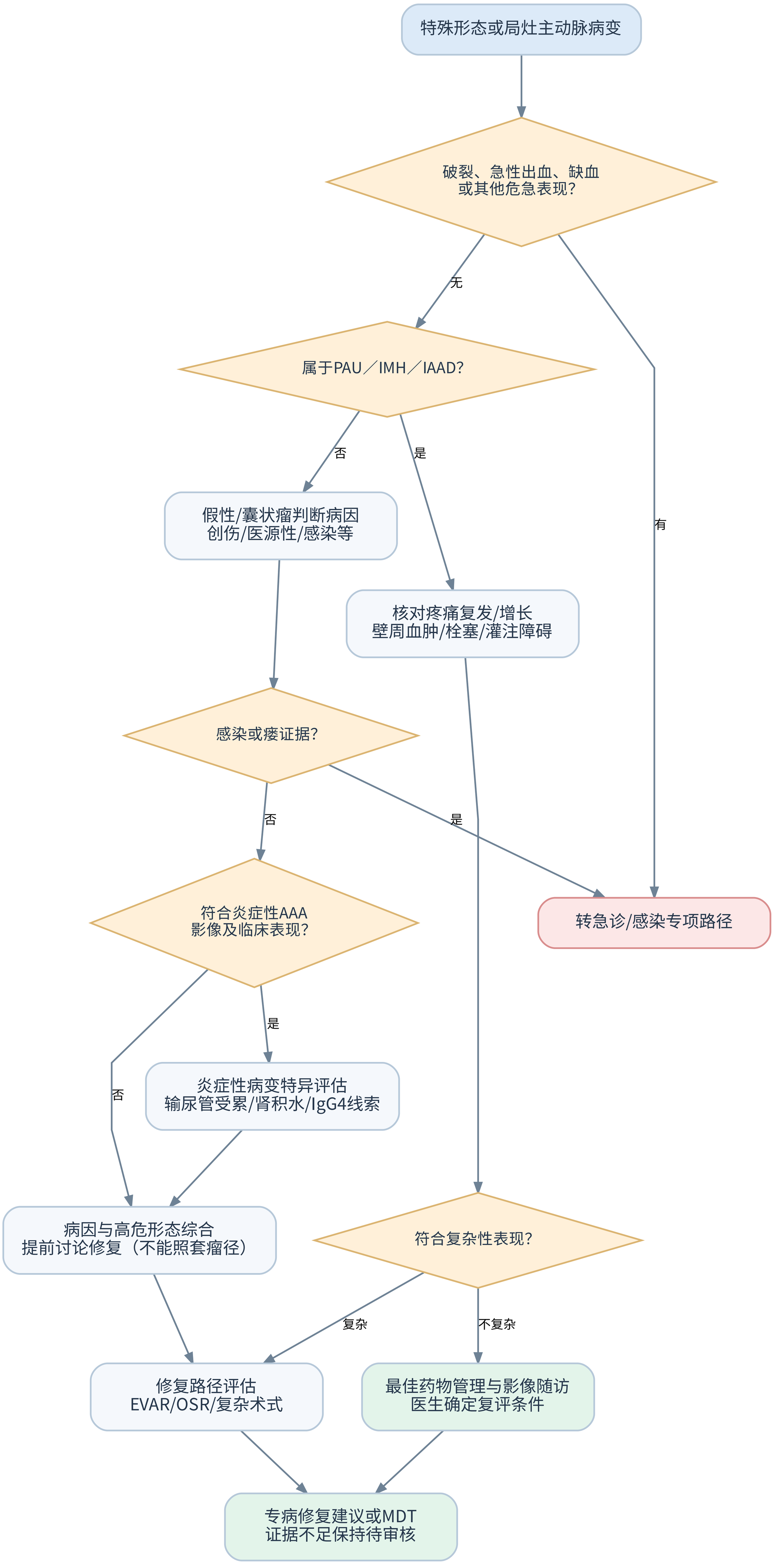
D3｜破裂或疑似破裂AAA详细路径



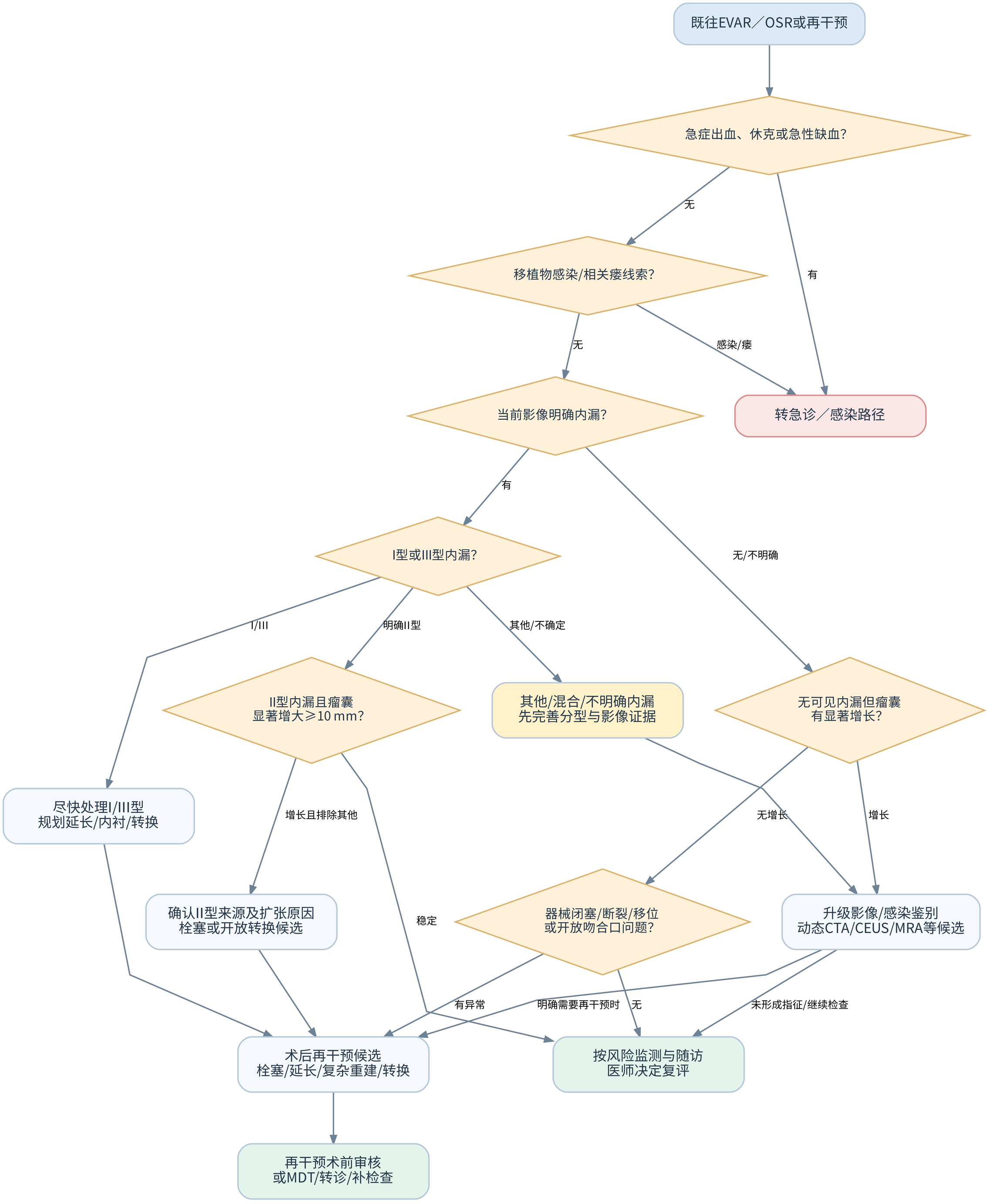
D4｜感染性主动脉疾病与移植物感染详细路径



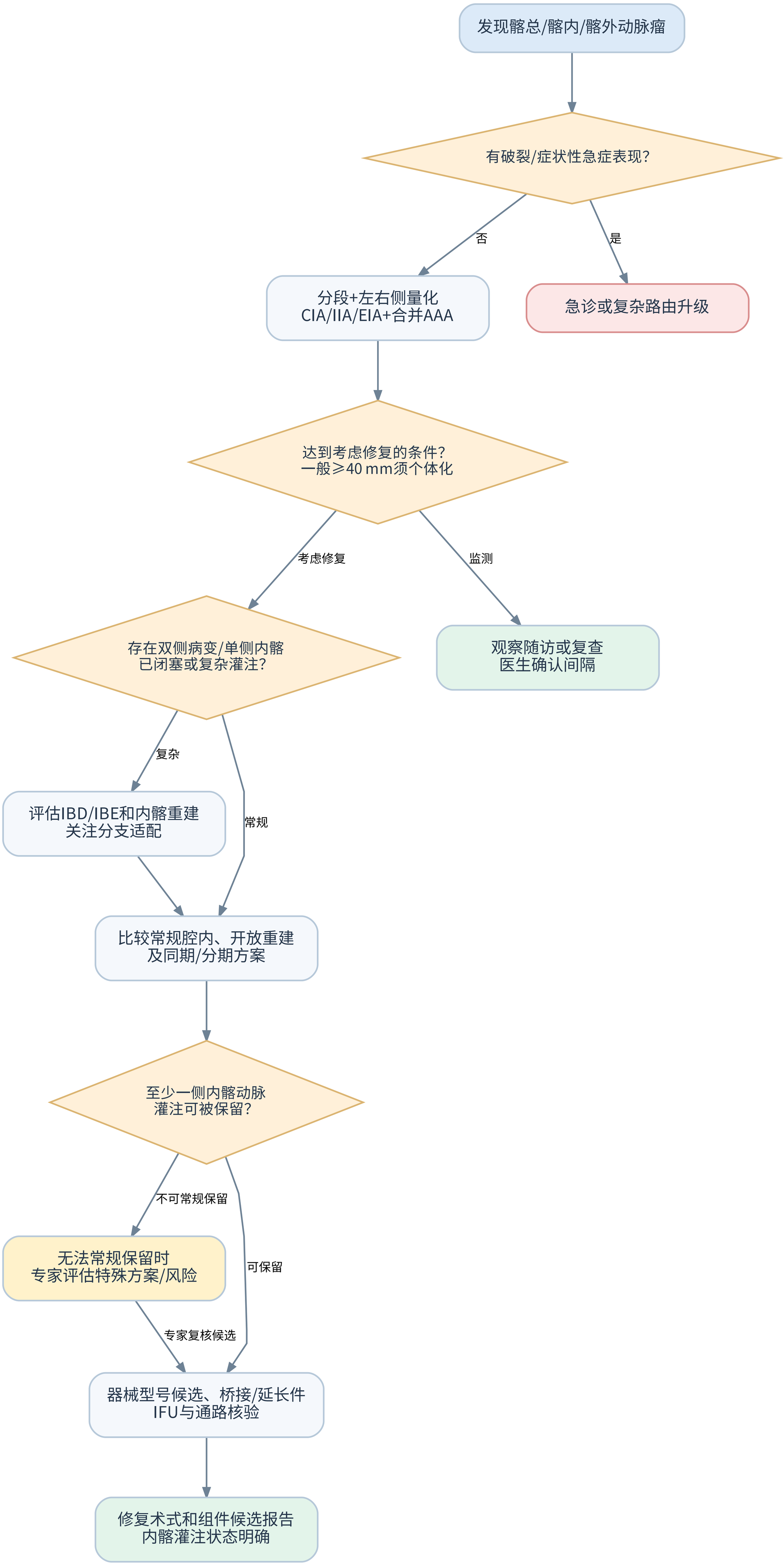
D5｜特殊主动脉疾病详细路径



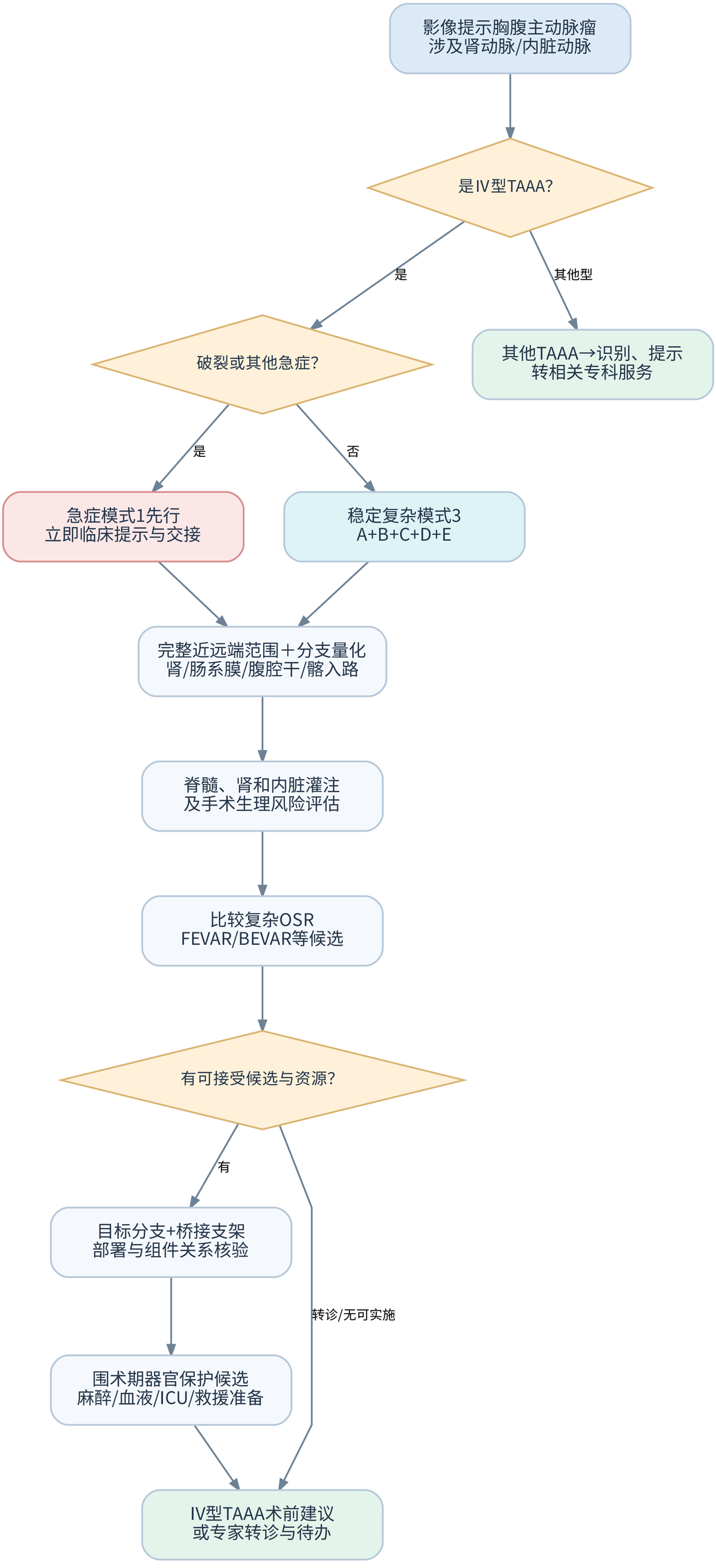
D6｜EVAR/OSR术后及并发症详细路径



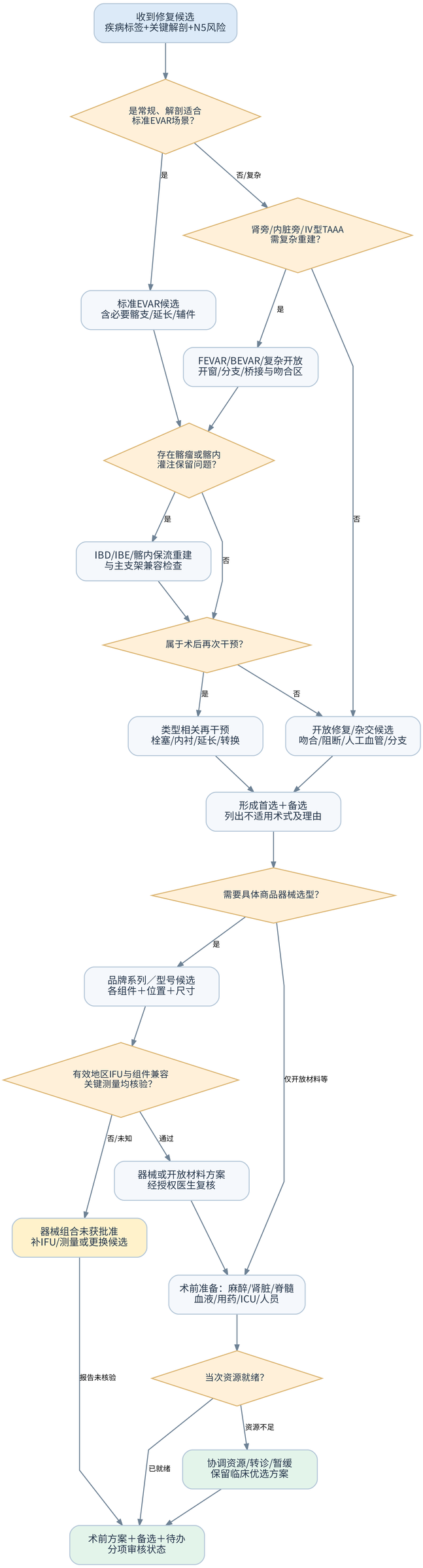
D7｜髂动脉瘤详细路径


发现髂总/髂内/髂外动脉瘤
有破裂/症状性急症表现?
否
是
分段+左右侧量化
CIA/IIA/EIA+合并AAA
急诊或复杂路由升级
达到考虑修复的条件?
一般≥40 mm须个体化
考虑修复
监测
存在双侧病变/单侧内髂
已闭塞或复杂灌注?
观察随访或复查
医生确认间隔
复杂
常规
评估IBD/IBE和内髂重建
关注分支适配
比较常规腔内、开放重建
及同期/分期方案
至少一侧内髂动脉
灌注可被保留?
不可常规保留
可保留
无法常规保留时
专家评估特殊方案/风险
专家复核候选
器械型号候选、桥接/延长件
IFU与通路核验
修复术式和组件候选报告
内髂灌注状态明确

IV型TAAA｜完整扩展路径



共享｜术式—器械—术前准备详细路径



共享｜异常回流与审批安全详细路径

